



Tests you can trust

Name : Hemant A Kikani E 381(25Y/M)

Date : 26 May, 2026

Test Asked : Aarogyam Winter Basic Package

Report Status : Complete Report



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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

Patient Name : **HEMANT A KIKANI E 381 (25Y/M)** Tests Done : **AAROGYAM WINTER BASIC PACKAGE**
Referred By : **SELF**
Address : **DYNATECH SYSTEM PVT LTD,B-18 TIMES CORPORATE PARK,THALTEJ
ROAD,NEAR SHILAJ CROSSING ROAD,AHMEDABAD-. - 380059**

Report Availability Summary

Note: Please refer to the table below for status of your tests.

 **11** Ready  **0** Ready with Cancellation  **0** Processing  **0** Cancelled in Lab

TEST DETAILS

REPORT STATUS

AAROGYAM WINTER BASIC PACKAGE	Ready 
CHLORIDE	Ready 
25-OH VITAMIN D (TOTAL)	Ready 
SODIUM	Ready 
TSH - ULTRASENSITIVE	Ready 
VITAMIN B-12	Ready 
HBA PROFILE	Ready 
HEMOGRAM - 6 PART (DIFF)	Ready 
LIVER FUNCTION TESTS	Ready 
IRON DEFICIENCY PROFILE	Ready 
KIDPRO	Ready 
LIPID PROFILE	Ready 

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Patient Name : HEMANT A KIKANI E 381 (25Y/M) Tests Done : AAROGYAM WINTER BASIC PACKAGE
Referred By : SELF
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ROAD,NEAR SHILAJ CROSSING ROAD,AHMEDABAD-. - 380059

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTE	49.7	%	20-40
TOTAL RBC	5.59	X 10 ⁶ /μL	4.5-5.5
IRON DEFICIENCY			
% TRANSFERRIN SATURATION	12	%	13 - 45
IRON	56	μg/dL	65 - 175
UNSAT.IRON-BINDING CAPACITY(UIBC)	393.6	μg/dL	162 - 368
LIPID			
HDL CHOLESTEROL - DIRECT	66	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	107.22	mg/dL	< 100
TC/ HDL CHOLESTEROL RATIO	2.8	Ratio	3 - 5
RENAL			
BLOOD UREA NITROGEN (BUN)	5	mg/dL	7.94 - 20.07
BUN / Sr.CREATININE RATIO	6.94	Ratio	9:1-23:1
UREA (CALCULATED)	10.7	mg/dL	Adult : 17-43
VITAMIN			
25-OH VITAMIN D (TOTAL)	11.7	ng/mL	30-100
VITAMIN B-12	139	pg/mL	211-911

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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PARK, THALTEJ ROAD, NEAR SHILAJ CROSSING
ROAD, AHMEDABAD-. - 380059

Sample Collected on (SCT) : 26 May 2026 10:36
Sample Received on (SRT) : 26 May 2026 15:01
Report Released on (RRT) : 26 May 2026 17:01
Sample Type | Barcode : SERUM | FQ024653

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL) Bio. Ref. Interval. :-	C.M.I.A	11.7	ng/mL
DEFICIENCY : <10 ng/ml INSUFFICIENCY : 10 - 30 ng/ml SUFFICIENCY : 30 - 100 ng/ml TOXICITY : >100 ng/ml			

Please correlate with clinical conditions.

Method:- Fully Automated Chemi Luminescent Microparticle Immunoassay

Tests Done : AAROGRAM WINTER BASIC PACKAGE

Report Remarks : Labcode:2605094862/GUJ28

Dr Margee Shah MD
(Path)

Dr Yukti Shah, MD
(Path)



Scan QR to verify (valid for
30 days from release time)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12 Bio. Ref. Interval. :-	C.L.I.A	139	pg/mL

Normal : 211 - 911 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:

Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569-73.

Please correlate with clinical conditions.

Method:- COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

Tests Done : AAROGRAM WINTER BASIC PACKAGE

Report Remarks : Labcode:2605094862/GUJ28

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	56	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	449	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	12	%
UNSAT. IRON-BINDING CAPACITY (UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	393.6	µg/dL
Please correlate with clinical conditions.			

Tests Done : AAROGRAM WINTER BASIC PACKAGE

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	185	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	66	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	107.22	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	52	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	2.8	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	0.79	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	1.6	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.61	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	118.73	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	10.41	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fastina is mandatorv for lipid parameters. If not. values might fluctuate.

Tests Done : AAROGYAM WINTER BASIC PACKAGE

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	76.96	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	1	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.18	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.82	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	15.42	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	18.1	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	15.04	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.2	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.67	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.66	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.01	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.55	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹ method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : AAROYAM WINTER BASIC PACKAGE

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mgshah

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Yukti

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TEST NAME	TECHNOLOGY	VALUE	UNITS
CHLORIDE	I.S.E - INDIRECT	102.78	mmol/L
Bio. Ref. Interval. : ADULTS: 98-107 MMOL/L			
Clinical Significance : An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).			
Method : ION SELECTIVE ELECTRODE - INDIRECT			
SODIUM	I.S.E - INDIRECT	139.66	mmol/L
Bio. Ref. Interval. : Adults: 136-145 mmol/l			
Method : ION SELECTIVE ELECTRODE - INDIRECT			
Please correlate with clinical conditions.			

Tests Done : AAROYAM WINTER BASIC PACKAGE

Report Remarks : Labcode:2605094862/GUJ28

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	5	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.72	mg/dL	0.72-1.18
BUN / Sr.CREATININE RATIO	CALCULATED	6.94	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	10.7	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	14.86	Ratio	< 52
CALCIUM	PHOTOMETRY	9.75	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	6	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : AAROGRAM WINTER BASIC PACKAGE

Report Remarks : Labcode:2605094862/GUJ28

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TSH - ULTRASENSITIVE	C.M.I.A	0.702	μIU/mL	0.35-4.94

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

USTSH - Fully Automated Chemi Luminescent Microparticle Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : AAROYAM WINTER BASIC PACKAGE

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	130	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : AAROGRAM WINTER BASIC PACKAGE

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Referred By : SELF Sample Received on (SRT) : 26 May 2026 15:07
Address : DYNATECH SYSTEM PVT LTD, B-18 TIMES CORPORATE Report Released on (RRT) : 26 May 2026 16:23
PARK, THALTEJ ROAD, NEAR SHILAJ CROSSING Sample Type | Barcode : EDTA Whole Blood | FD456616
ROAD, AHMEDABAD-. - 380059

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.2	%

Bio. Ref. Interval. :

As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
≥ 6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
≥ 8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 103 mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE, HEMOGRAM

Report Remarks : Labcode:2605095543/GUJ28


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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	15.8	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	50	%	40.0-50.0
Total RBC	HF & EI	5.59	X 10⁶/μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	89.4	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	28.3	pq	27.0-32.0
Mean Corp. Hemo. Conc (MCHC)	Calculated	31.6	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	42	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.7	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	203.1	-	*Refer Note below
MENTZER INDEX	Calculated	16	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	5.09	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	40.6	%	40-80
Lymphocytes Percentage	Flow Cytometry	49.7	%	20-40
Monocytes Percentage	Flow Cytometry	6.1	%	2-10
Eosinophils Percentage	Flow Cytometry	2.8	%	1-6
Basophils Percentage	Flow Cytometry	0.6	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.07	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.53	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.31	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.14	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	234	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	11.4	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	13.4	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	34.3	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.27	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets: Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedance, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

Tests Done : HBA PROFILE, HEMOGRAM

Report Remarks : Labcode:2605095543/GUJ28

Dr Margee Shah MD
(Path)

Dr Yukti Shah, MD
(Path)



Scan QR to verify (valid for 30 days from release time)

CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
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EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on **022-3090 0000**

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* T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),

* As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited